



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ولاء على اسماعيل يونس

العنوان : 0

الحي / المدينة : الهرم

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 27507022100067 رقم التليفون : 01143946916

العمر (تاريخ الميلاد) : 02-07-1975 المهنة : ربه منزل

### الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان الانسجه الرخوه

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان الانسجه الرخوه

خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6564] قسم العلاج : الاورام

التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ..... التوقيع: .....  
الاسم: د/ندى حسن الوظيفة: ..... التوقيع: .....  
الاسم: د/نرمين رمضان الوظيفة: ..... التوقيع: .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : ..... قسم : .....  
جهة العلاج : ..... نفقات : .....  
ملاحظات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

|  |
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|  |
|--|

|                                                                          |                                                                                     |
|--------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> ولاء على اسماعيل يونس<br><b>National ID:</b> 27507022100067 |  |
|--------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بأن نوضح فيما يلي تقريراً عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد أنه |
|----------------------------------------|

|                                                                          |
|--------------------------------------------------------------------------|
| A patient of soft tissue sarcoma for Supportive and palliative Treatment |
|--------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |

١١٤٣٩٤٦٩١٦

جمهورية مصر العربية  
بطاقة تحقيق الشخصية

ولاء  
على اسماعيل يونس  
٣٢ سن عمرين الخطاب-امام فندق اوربا  
الهرم - الجيزة

١٩٧٥/٠٧/١٤  
٢٧٥٠٧٠٢٢١٠٠٠٦٧  
KD8398540

٢٠٢٣/٠٩ ٢٧٥٠٧٠٢٢١٠٠٠٦٧  
حاصله على دبلوم تجارة  
انثى مسلمة  
احمد محمد على السيد احمد  
البطاقة سارية حتى ٢٠٣٠/٠٩/٠٣

١

|                                                 |                                                |
|-------------------------------------------------|------------------------------------------------|
| Patient Name : الأستاذة / ولاء على اسماعيل يونس | Request No. : 13004230                         |
| Patient No. : 27507022100067                    | Request Date : 29/12/2025                      |
| Path No. : 4020/25                              | Verification Date : 04/01/2026                 |
| Sex : Female                                    | Financial Category : جوسفاف روسي - نفقة الدولة |
| Age : 50 Y                                      | Department : Out Patient                       |
| Referring Doc :                                 | Registered by : marwa mohamed youss            |

### Pathology Report

#### Addendum Report.

Clinical Data: Right thigh mass.

Nature of specimen: Tru-cut biopsy, collected on 29/12/2025, at 4:45 p.m, cold ischemia time: 10 seconds.

#### Gross examination:

Three grey white tissue cores ranging in length from 0.5 cm to 1 cm long, totally submitted.

#### Technique:

Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; SMA clone (HHF35), Desmin clone (D-R-11) & CK clone (AE1/AE3).

#### Microscopic examination:

Histopathological examination revealed cores of tumor formed of malignant growth showing spindle and epithelioid cells with hyperchromatic nuclei, high N/C ratio and marked degree of pleomorphism with few multinucleated giant cells. There is intervening stromal ectatic vessels with associated extensive necrosis and scattered mitotic figures.

#### IPT (All controls show appropriate reactivity):

- SMA: Negative in tumor cells.
- Desmin: Focal scattered positive cells.
- CK: Negative in tumor cells.

#### Diagnosis

Right thigh mass, Tru-cut biopsy:

- High grade pleomorphic sarcoma.
- Differential diagnosis includes; 1) Undifferentiated pleomorphic sarcoma., 2)

Verified by :

Dr.Heba Hasan



|                                                  |                                                |
|--------------------------------------------------|------------------------------------------------|
| Patient Name : الأستاذ / ولاء على اسماعيل يونس : | Request No. : 13004230                         |
| Patient No. : 27507022100067                     | Request Date : 29/12/2025                      |
| Path No. : 4020/25                               | Verification Date : 04/01/2026                 |
| Sex : Female                                     | Financial Category : جوستاف روسي - نفقة الدولة |
| Age : 50 Y                                       | Department : Out Patient                       |
| Referring Doc :                                  | Registered by : .marwa mohamed youss           |

### Pathology Report

Dedifferentiated liposarcoma., 3) Malignant peripheral nerve sheath tumor (MPNST)., 4) Rhabdomyosarcoma., 5) Leiomyosarcoma.

\*\* Further immunohistochemistry is needed to confirm the nature of the tumor including (H-caldesmon, Myogenin, TLE-1, CDK4, MDM2, S100, SOX-10,.....).

Signature  
Prof. Dr. Maha Guimei.

Verified by :  
Dr.Heba Hasan

Printing Date : 22/04/2026 14:34:15

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

7747 DMS

Name : Walaa Ali Esmaeel Yones

Date : 25-01-2026 02:26

Branch : Portable

Medical No. : GRI-27507022100067

Ref.Physician Prof : MRI RT HIP

Ins.Company :

## MRI EXAMINARION OF BOTH HIP JOINTS

### Technique :

- CORONAL T1, T2
- Axial T1 , T2, STIR
- SAGITTAL T2
- Post contort series

### Findings:

- Right upper thigh medial compartment large fairly defined complex cystic/solid mass lesion is seen with multiple internal spetation displaying heterogeneous low T1 and isointense T2 with areas of of low T1 and bright T2 signal with post contrast enhancement of solid component and peripheral , measuring about 15.7x15x24.7cm in its AP, TS and CC dimensions respectively.
- Maintained normal articular relation of both hip joints.
- Preserved normal spherical configuration of both femoral heads.
- No gross hip joint effusion.
- Normal marrow signal of the examined bones.

### OPINION:

- Right upper thigh malignant looking large fairly defined complex cystic/solid mass suggestive of neoplastic nature for histopathological correlation

*Dr. Nourhan Mohamed,MD*

*Dr. Nourhan Mohamed,MD*

Name : Walaa Ali Esmaeel Yones

Date : 25-01-2026 02:26

Branch : Portable

Medical No. : GRI-27507022100067

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### Findings:

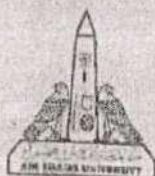
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- Right upper thigh malignant looking large fairly defined complex cystic/solid mass suggestive of neoplastic nature for histopathological correlation

*Dr. Nourhan Mohamed,MD*

*Dr. Nourhan Mohamed,MD*



|              |                       |     |                    |     |            |
|--------------|-----------------------|-----|--------------------|-----|------------|
| Patient Name | ولاء على اسماعيل يونس | PID | 10210124283<br>489 | D/T | 15-01-2026 |
|--------------|-----------------------|-----|--------------------|-----|------------|

**C.T. SCAN OF THE ABDOMEN AND PELVIS:-**

*Axial contiguous cuts were taken through the abdomen and pelvis after I.V. contrast injection.*

- ✍ The right gracilis muscle is seen occupied by large heterogenous mixed solid and cystic mass (predemonently cystic ) with areas of
- ✍ Right thigh is seen enlarged asymmetrically by virtue ,the medial aspect of the thigh shows large intramuscular mixed cystic solid predomentaly cystic mass
- ✍ The liver is of average size displaying normal homogenous CT density with no abnormal focal lesions.
- ✍ No extra or intra-hepatic biliary dilatation.
- ✍ The spleen is of average size displaying normal and homogenous CT appearance.
- ✍ The pancreas shows normal C.T. appearance.
- ✍ Both kidneys are of average size being functioning.
- ✍ Normal size and C.T. density of the adrenal glands.
- ✍ No abdominal adenopathy.
- ✍ No ascites.
- ✍ Normal appearance of the urinary bladder with normal wall thickness and no calculi.
- ✍ Normal C.T. appearance of the uterus and ovaries.
- ✍ Normal CT appearance of the prostate and the seminal vesicles.
- ✍ Normal appearance of the rectum, and recto sigmoid junction.
- ✍ No pelvic fluid collection.
- ✍ No pelvic adenopathy.

**IMPRESSION:-**

- ✍ Normal CT examination of the abdomen and pelvis.

- ✍ The spleen is of average size displaying normal and homogenous CT appearance.
- ✍ The pancreas shows normal C.T. appearance.
- ✍ Both kidneys are of average size, with no back pressure nor stones.
- ✍ Normal size and C.T. density of the adrenal glands.
- ✍ No abdominal adenopathy.
- ✍ No ascites.
- ✍ Normal appearance of the urinary bladder with normal wall thickness and no calculi.
- ✍ Normal C.T. appearance of the uterus and ovaries.
- ✍ Normal appearance of the rectum, and recto sigmoid junction.
- ✍ No pelvic fluid collection.
- ✍ No pelvic adenopathy.
- ✍ The lower chest cuts shows no pleural effusion nor nodules.

#### IMPRESSION:-

- ✍ *Hepatic focal lesion likely focal fatty infiltration yet malignant possibility cannot be excluded for further assessment by MRI dynamic*
- ✍ *Right thigh adductor compartment malignant lesion as described*
- ✍ The adductor compartment lesion is butting the GSV at likely mid thigh with no clear line of separation, for better assessment with US
- ✍ Few rounded suspicious looking inguinal LNs are noted (likely 3)

Asmaa Nabil, MSc

# GUSTAVE / ROUSSY /

INTERNATIONAL  
EQUIPE

Date Of Submission:

Cbl  
9-11

Date Of MDT:

09/13/2026 14:43:27

## Medical Report

|                     |                                                                    |                  |                                                                                           |
|---------------------|--------------------------------------------------------------------|------------------|-------------------------------------------------------------------------------------------|
| Patient Name :      | Walaa Ali Ismail Younes                                            | Age :            | 51 Y                                                                                      |
| Sex :               | <input type="radio"/> Male <input checked="" type="radio"/> Female | Comorbidity :    | 27507022100067                                                                            |
| ID No. :            | 6646                                                               | Telephone :      | 01015764219                                                                               |
| Family History :    |                                                                    | Marital Status : | <input type="radio"/> Married <input type="radio"/> Divorced <input type="radio"/> Single |
| Menopausal Status : |                                                                    | Reason of MDT :  |                                                                                           |
|                     |                                                                    | MDT name::       |                                                                                           |

## Medical Report

Diagnosis : RT proximal thigh STS - malignant spindle &amp; epithelioid neoplasm.

Notes :

History:

RT proximal thigh STS - malignant spindle &amp; epithelioid neoplasm.

Baseline MRI RT thigh (pre-CT):

Large complex solid mass involving medial compartment of RT upper thigh, measuring 15.7 x 15 x 24.7 cm.  
Hip joints preserved; no gross joint effusion.  
No BM involvement.

Initial CT CAP:

B/L pulmonary nodules, largest ~14 x 12 mm, suspicious for MTS.  
Hypodense hepatic S4 lesion ~22 x 26 mm, suspicious for MTS.

Received :

-Radiotherapy then  
-Adriamycin + Ifosfamide (received 6cycles last 30/7/2026 )  
-Vacuum for wound management.

Post MRI RT thigh:

Marked post-op/post-CT changes within the operative bed.  
Multiple residual scattered intramuscular soft-tissue lobulated lesions, largest ~2.9 x 2.6 x 1.9 cm.  
No BM involvement.  
RT thigh muscle atrophy.

Post-CT CAP:

Significant regression of B/L pulmonary nodules; largest now ~7.5 mm vs ~14 x 12 mm initially ??  
Previously seen hepatic lesion is no longer visualized ???  
No new definite visceral MTS.

Investigations:

Recommendations:

MDT DECISION:

NEW HFL FOR US GUIDED BX  
MEANWHILE CONTINUE IFOSFAMIDE SINGLE AGENT FOR 2 CYCLES

Specialty

Surgery

Pathology

Radiology

about:blank

Cbl 9-11